

Analysis of Caregiver Burden in South Asian Families in the Dallas-Fort Worth Metropolitan Area: Insights for Social Practice.

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ABSTRACT

This study examines the factors that influence elder caregiver burden in South Asian families in the Dallas-Fort Worth area. A sample survey of 118 households with an elderly person receiving care was conducted. Data were gathered from primary caregivers via an interview survey. The results suggest that role demand is a significant factor in understanding the phenomenon of caregiver burden in multigenerational South Asian households. As role demand increases, the amount of caregiver burden increases. However, factors such as Asian cultural norms and size of support network tend to reduce the level of role demand. Implications for social service programs and policy are discussed.

One of the most notable demographic trends in the United States today is the rapid aging of its population. This process is affecting all sections of society, irrespective of ethnic, national, and other distinctions. Population aging is often accompanied by an increase in the perceived burden on caregivers. Once considered to be the responsibility of the family, this role is intended to assist the elderly to achieve a sense of well being, despite the deterioration of their physical and mental abilities.

As the size of the non-institutionalized elderly population in the United States has increased, the physical and psychological consequences of providing care have become pervasive. Among these consequences are various sources of stress, several of which have been identified by researchers (Litwin, 2003; Pinquart and Soreson, 2003; Baille, Norbeck, and Barnes, 1988). These include subjective feelings of abandonment and a sense of entrapment. Such feelings have been labeled "caregiver burden" (Baille, Norbeck, and Barnes, 1988; Brewer, 2000; George and Gwyther, 1986, Zarit, Todd and Zarit, 1986).

Caregiving for the elderly ordinarily takes place in nursing homes and within the family. In the latter case, adult children typically provide care within the same household. The rapidly increasing immigrant, ethnic-minority population in the US is predominantly composed of family types with extended households of adults spanning multiple generations. Thus, a high proportion of the elderly in these groups is likely to be cared for in such multigenerational settings.¹

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During the past three decades the number of immigrants in the United States has increased considerably. Nearly two thirds of these immigrants have come from Mexico, Cuba, El Salvador, Guatemala, Nicaragua, the Dominican Republic, Haiti, the Philippines, Vietnam, South Korea, China, Taiwan, and the Indian sub-continent (Pedraza and Rumbaut, 1996). South Asians constitute the third largest group within the Asian and Pacific Islander populations (Barnes and Bennett, 2002). Passage of the Immigration Reform Act of 1965 stimulated an increase in the number of South Asian immigrants during through the 1970s. The provisions of this reform enabled highly qualified professionals from developing nations to immigrate to the United States. The parents and relatives of these immigrants constituted a second wave of South Asian immigrants under the Family Reunification Act (Gonzales, 1993). Most of these people were poorly educated elderly parents and relatives of the earlier wave. However, they were not physically frail when they arrived in the U.S.

For the first generation of South Asian households, having elderly parents in the United States meant an increased dependency on adult children. This was a nearly universal phenomenon. Cultural values emphasized taking care of elderly parents and respect for the aged (Kyu-taik, 2001; Wilmoth and Chen, 2003), which led to the common practice of forming multi-generational households (Bengston, Rosenthal and Burton, 1996; Gangrade, 1989; Gupta, 1996). Under these circumstances, the provision of care for elders contributed to heightened stress, poor mental health, frustration, and feelings of hopelessness (Chandras, 1997).

The research reported here tests select theoretical propositions on the impact of sociocultural factors on the caregiver burden experienced by first generation South Asian immigrants in the Dallas-Fort Worth Texas metropolitan area.² Several issues underscore the importance of studying this “caregiver burden, the most important of which is the close association between the high levels of caregiver burden and mental illness. Asians in general are noticeably reluctant to talk about these mental health issues because they tend to be ashamed to reveal that they experience stress over caregiving-related problems. Therefore, problems often remain hidden until they become serious (U.S. Department of Health and Human Services, 2001). South Asian immigrants tend to under-utilize traditional mental health services, despite the easy accessibility of such services and their serious need for them (Mokuau, 1987; Ramisetty-Mikler, 1993; Gupta, 1996).

The literature on elder care in South Asian families emphasizes the significance of cultural factors in predicting the levels of caregiver burden (Bose and Gangrade, 1988; Desai, 1982; Gupta, 1996; Gupta and Pillai, 2002). These factors are defined by two indicators: the extent of social support networks (Antonucci and Sherman, 1997, Gupta and Pillai, 2002), and the relevance of filial piety norms of elder care (Ching Ying Ng; Phillips and Keng-mun Lee, 2002; Gupta, 2000). For those interested in providing effective mental health services to caregivers of the elderly, it is necessary to understand both the cultural and social components of this phenomenon (Mills and Henretta, 2001). Unfortunately, studies that attempt to delineate the nature of the relationship between cultural factors and caregiver burden in South Asian families are few and far between. The study discussed in the remainder of this article seeks to shed light on this relationship.

CONCEPTUAL FRAMEWORK

Theories of elder caregiver burden generally focus on three aspects: First, the caregiver burden increases with an increase in the scope of caregiving tasks. Second, as family members attempt to fulfill obligations and responsibilities associated their roles -- in addition to that of elder caregiver, they experience an excess of conflicting obligations (Gupta, 1996; Hyde, et al, 1995). Third, because the family is a web of reciprocal relationships, the burden is decreased when elders carry out household tasks.

South Asian cultural influences also shape the amount of elder care provided by family members (Bose and Gangrade, 1988; Desai, 1982; Gupta and Pillai, 2002). The literature on elder care in South Asian households in particular suggests two indicators of cultural influences. One is the extent of social support networks (Antonucci and Sherman, 1997; Gupta and Pillai, 2002). The other is the relevance of filial piety norms of elder care in performing the caregiver role (Ching Ying Ng; Phillips and Keng-mun Lee, 2002; Gupta, 2002).

Based on these considerations, the model proposed in this study suggests that the effect of cultural factors on caregiver burden is mediated by three well-known determinants. These are: (1) the scope of the care giving tasks, (2) the extent of demands of competing roles, and (3) the amount of support provided by the elders themselves. We have developed three explanations, each providing a description of the theoretical relationship between caregiver burden and one of the three factors.

One explanation of caregiver burden focuses on the resources caregivers bring to their role. The amount of time and energy required to provide care is likely to be influenced by the number of economic and social roles of that caregivers perform. As the number of roles increases, caregivers are likely to experience "role demand" (Goode, 1960; Gupta, 1996; Sieber, 1974). This term indicates the extent of role conflict and role overload (Zhohar, 1997). Role conflict is defined as the degree to which a caregiver experiences conflicting expectations in the setting within which care is provided (Franks and Stephens, 1992; Marks, 1977; Sieber, 1974; Soldo and Myllyluoma, 1983; Troll, 1987). As the extent of caregiving activities increases, the likelihood that the caregiver will experience both role conflict and role overload also increases.

- Hypothesis 1: As the extent of caregiving tasks increases, the amount of role demands increases.
- Hypothesis 2: As the amount of role demand increases, the amount of caregiver burden is likely to increase.

South Asian immigrants bring with them strongly family-oriented cultural norms and expectations. These deep-rooted beliefs are the primary reasons why they provide care for their parents and relatives. Members of this group, especially first generation immigrants, possess several common cultural characteristics. These include feelings of respect and duty toward elders (Asian filial piety) and involvement of kin and friends in caregiving (size of social support networks; Bose and Gangrade, 1988; Desai, 1982; Gupta, 2002).

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- Hypothesis 3: The more strongly that individuals and families identify with Asian filial piety norms, the greater will be the size of their social support groups.

Social support groups are often based on kinship (Bose and Gangrade, 1988; Gupta, 2002), whereas individuals who have internalized ethnic cultural norms are likely to be involved in informal support networks (Desai and Coelho, 1980; Gupta, 1996). Support from friends, relatives, and next-of-kin increases the amount of emotional and labor resources available to the household. These, in turn, are likely to reduce the extent and amount of role demand (Gangrade, 1989).

- Hypothesis 4: As the size of social support group increases, the amount of role demand is likely to decrease.

As noted, caregiving among South Asians in the United States is a product of cultural expectations, duty, love, and a positive attitude toward aging in general (De Souza, 1982). Hindu teachings maintain that individuals are born with an obligation to fulfill three types of duties or *Runas*. The first is called *Deva Runa*, duty toward God. The second is *Rishi Runa*, duty toward wise sages. The third is *Pitra Runa*, duty toward parents and ones ancestors. The duty of children toward their parents stems from the fact that parents make enormous economic and emotional investments in them. Children are therefore obliged to make investments necessary to enhance their parents' well being in their old age. Over a lifetime, reciprocal relationships are carefully cultivated by both parents and children in order to realize the cultural ideal of assuming responsibility of caring for parents in their old age (Sung, 1990). In Islam, serving one's parents is considered sacred and it is the right of the parents' to expect it. Expressing any irritation when the elderly become difficult to handle is taken as a sign of irresponsibility. Because of the importance attached to caregiving to elders, household functions and tasks are often rearranged and managed to minimize role overload and conflict (Chowdhari, 1993).

- Hypothesis 5: As identification with Asian filial piety norms increases, the amount of role demand is likely to decrease.

A second explanation offers an explicit and simple reason for changes in caregiver burden. The most influential determinants of caregiver burden are the tasks entailed in caregiving (Gold et al., 1994; Yates et al., 1999). The amount and type of assistance the caregiver provides a measure of the intensity of caregiving. Caregivers often contribute selflessly toward enhancing the lives of the elders (Chowdhari, 1993).

- Hypothesis 6: As identification with Asian filial piety norms increases, the extent of caregiving tasks increases.
- Hypothesis 7: As the extent of caregiving tasks increases, the amount of caregiver burden increases.

A third explanation of caregiver burden focuses on the support provided by the elderly family member. Social exchange theory has been used to investigate exchanges of family care between the care recipients and caregivers, as well as the degree of reciprocity in support relationships (Dowd, 1975; Gupta, 2002; Liang et al., 2001; Van Tilburg and Broese van Groenou, 2002). The reciprocal nature of social support also shapes the roles of elders as family members and care receivers (Becker et al, 2003; Gupta, 2002; Thibaut and Kelley, 1959; Walker, Martin, and Jones, 1992). Interdependency among generations with regard to caregiving is a common phenomenon in India (Ramanathan and Ramanathan, 1994). In general, recipients of care are likely to reciprocate with their own support for the caregiver as the number of familial and extra familial activities of the caregiver increases. Elderly parents often engage in activities to reduce the overall role demands on the primary caregivers (Boerner and Reinhardt, 2003; Gupta, 2002).

- Hypothesis 8: As role demand increases, the amount of support provided by the elder is likely to increase.
- Hypothesis 9: As the amount of support provided by the elder increases, the amount of caregiver burden is likely to decrease.

Broadly, the proposed model of caregiver burden among South Asian families suggests that caregiver burden is a function of the extent of caregiving tasks performed by the primary caregiver, the amount of support provided by the elders, and the role demand experienced by the primary caregiver. Role demand intervenes in the relationship between the size of the social support network, and identification with Asian filial piety norms and the amount of caregiver burden. A schematic representation of the hypothesized relationships in the proposed model is presented in Figure 1.

METHODS AND MATERIALS

Because caregiving is an important function in South Asian families, it is usually entrusted to people thought deemed old enough to be mature and responsible. Within an age range of 21 to 29 years, a family member is expected to start a family and undertake the responsibilities of supporting parents, financially and emotionally if needed (Pillai, 1985). For this study, the midpoint of this age range was used to select the sample. Thus, the term "caregiver" refers to a person of about 25 years of age who is identified by a family member in a multi-generational household as the person who provides at least 4 hours of care per week. This person assists the care recipient in at least one "activity of daily living" (ADL) or two "instrumental activities of daily living" (IADL).

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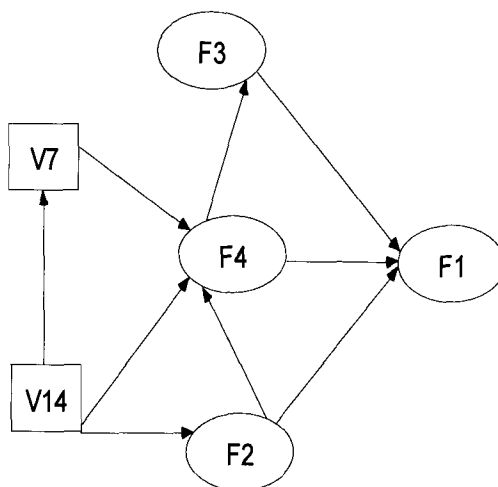


Figure 1: Proposed model of elder caregiver burden in South Asian Families

Key: Latent Variables: F1= Perceived Caregiver Burden: indicators v1 to v4; F2= Amount of caregiving tasks : indicators v6 and v7; F3 = Amount of support provided by elder; indicators v9 to v11; F4 = Amount of role demand : indicators v12 and v13. Measured variable; v14 = Identification with Asian filial piety norms; v17 = Size of the social support net work; v1= Impact on finances; v2 = Feelings of abandonment; v3 = Impact on work schedule; V4 = Sense of entrapment; v6 = Amount of activities of daily living; v6 = Amount of instrumental activities of daily living; v9 = Emotional support by elder; v10 = Tangible support by elder; v11 = Informational support by elder; v12 = Role conflict; v13= Role overload

We have assumed that about one half hour per day is needed to assist with one ADL or two IADLs. In households with more than one caregiver, one person self-identified himself/herself as the one undertaking most of the responsibility for providing care to the elder. This selected member is referred to as the "primary caregiver." The care recipient is either a parent or parent-in-law, at least 60 years of age who had lived in a multi-generational South Asian household for at least one year. The age of 60 is of social significance among South Asians, as it connotes the beginning of the last phase of life. During this phase one is expected to dissociate gradually from the economic and social world in preparation for death (Chakravarthi, 1995; Kurian, 1995). Because many

households include more than one care recipient, oldest recipient was selected for the study (assuming that an older person will require more care than a younger person).

The first phase of the study involved developing a list of households in the Dallas-Fort Worth area that included one or more elderly persons receiving care from family members. Caregivers and elders who congregated at places of worship, such as the temple in the Dallas-Fort Worth area, were approached and their participation in the study was requested. In order to gather the elderly at a pre-specified venue, the principal author conducted educational sessions at several places of worship. These sessions provided information on such topics as social services for the elderly and various aspects of immigration laws. The caregivers and care recipients who participated were encouraged to provide names and addresses of caregivers of elders who were known to them and who resided in the Dallas-Fort Worth area. In addition, community leaders were approached to identify potential subjects. This procedure resulted in an address list of approximately five hundred households that matched our criteria.

A random sample of 200 households was drawn from this list, which included members of four religious groups Hindu, Muslim, Sikh, and Christian. Of the 200 households contacted, 150 agreed to participate in the study. The subjects were read a script for the telephone interview, and a verbal informed consent form was read out over the phone. Two bilingual, bicultural (Hindi/English) professors translated the interview guide into Hindi (which is understood by a large portion of South Asians). The Hindi questionnaire was then translated into English by two other bicultural and bilingual professors. Respondents were encouraged to choose an interview period when they would be least disturbed by the rest of the family. Periods during which the rest of the family was absent were particularly preferred. Of the 150 interviews conducted, 118 had no missing data. The remaining interviews included partial or incomplete responses.³

Operationalization of Variables

This study uses several scales to measure the concepts in the proposed model. All items in the scales were scored in such a way that low values indicate low levels of the concept under measurement and high values suggest high levels. The caregiver burden instrument was developed by Stommel, Given, and Given (1990) and consists of 31-items. The items are distributed among four sub-scales: (1) financial, abandonment by family, (2) impact on schedule and on health, and (3) sense of entrapment. Caregiving tasks are measured by an instrument developed by Strawbridge (1991). It has two sub-scales, one of which measures ADL and the other IADL. Fifteen items related to ADL and IADL tasks were selected from Strawbridge's scale. The concept "support provided by elder" was measured using parts of an instrument developed by Krause and Markides (1990). Only questions related to support provided by the elder were adopted for this study. The instrument has three sub-scales.

The construct, "role demand," is a latent variable with two indicators: role overload and role conflict. Four indicators were derived to measure role overload and five indicators were constructed to measure role conflict. Indicators of role conflict were developed using questions related to whether caregiving for the elderly affected caregivers' daily tasks and

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other relationships (Pearlin et al., 1990). The questions were related to levels of role conflict and role overload. The responses to the questions were measured on a scale ranging from 3 (serious problem) to 1 (not a problem). The concept "identification with Asian filial piety norms" was measured using the Asian filial piety instrument (Ho and Lee, 1974), which was designed to measure the caregivers' beliefs concerning cultural duty.

The data were analyzed using structural equation model (SEM) analysis as discussed by Bentler (1992).⁴ SEM analysis provides estimates of the strength of the relationships among the latent dimensions of the model. The analysis yields several measures that assess the overall fit of the proposed model to the covariance structure of manifest variables.

Results

Table 1 presents the demographic characteristics of caregivers and care recipients. The mean age of care recipient is approximately 71 years and about 85 percent of them had

Table 1 Demographic Characteristics of Caregivers

Variable	%	n	Variable	%	n
Caregiver Gender			Relationship to the Elder	20.3	24
Female	60.2	71	Daughter	33.9	40
Male	39.8	47	Son	38.1	45
Marital Status			Daughter-in-law	3.4	4
Married	89.8	106	Son-in-law	4.2	5
Never Married	5.9	7	Others		
Divorced	4.2	5	Family Income		
Education of the Caregiver			Less than \$10,000	23.7	28
Some College (13-14)	44.9	53	\$10,000-40,000	50.8	60
College (16-18)	35.6	42	\$41,000-70,000	11.0	13
Other	19.5	23	\$71,000-100,000	11.9	14
Number of Years in U.S.			Over \$100,000		
1-4	10.2	12	Caregiver Religion		
5-8	14.4	17	Hindu	64.4	76
9 and over	75.4	89	Muslim	19.5	23
			Christian	9.3	11
			Sikh	6.8	8

Table 1 (Continued)

Variable	%	n	Variable	%	n
Elder Live-in Arrangement			Years Elder in U.S.		
With Caregiver	83.1	98	1-4	84.7	100
Shared	14.4	17	5 and over	15.2	18
Other	2.4	3			
			Sample Characteristics	Mean	
Elder Gender			Elder Age	70.7	118
Female	51.7	61			
Male	48.3	57			
Elder Marital Status					
Married	45.8	54			
Widowed	54.2	63			
Other	0.8	1			

lived in the U. S. less than five years. The income range for the modal family was between 41 and 70 thousand dollars per annum. Caregivers averaged 30 years younger than the care receiver. Slightly more than 80 percent of the caregivers had some amount of college or higher education. Ninety percent of them were married and approximately 75 percent of them had lived in the U S for nine or more years. Nearly 59 percent of the caregivers were women --either daughters or daughters-in-law of the elderly care recipients.

Table 2 presents four sets of statistical measures. These are: (1) reliability scores (Cronbach's Alpha), (2) the factor loadings of indicators of latent constructs such as role demand, (3) the non standardized and standardized effects of exogenous latent factors on endogenous latent constructs such as "caregiver burden," and (4) R-squared values and measures of model fit. In general, the instruments have adequate reliability, with Alpha values of 0.6 or above. The factor loadings of all indicators related to the instruments are significant and have values greater than 0.3. The model explains approximately 71 percent of the variance in caregiver burden.

A statistically *insignificant* chi-square measure indicates a good fit of the model to the data. But the chi-square value of the proposed model turned out to be statistically significant is (91.4; df=58, $p < .005$). This is a substantial improvement over the independent model chi-square value (465.8; df=78; $p < .005$). Nevertheless, because the Comparative Fit Index (CFI) exceeds the recommended value of .9, we have concluded that the overall model fit is adequate.

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Table 2 Reliability Coefficients and Factor Loading of Indicators in the Model

Instrument (Dimensions)	Indicator	Alpha	Factor Loading
Caregiver Burden	Financial Burden	0.87	0.367
	Abandoned by Extended Family	0.89	0.556
	Impact on Work Schedule	0.90	0.906
	Feeling of Entrapment	0.97	0.593
Caregiving Tasks	ADL	0.87	0.314
	IADL	0.64	0.184
Support by Elder	Tangible Support	0.60	0.436
	Emotional Support	0.92	0.326
	Information Support*	0.83	1.000
Role Demand	Role Conflict*	0.91	1.000
	Role Overload	0.89	0.441
Identification with Asian Filial Piety Norms	Filial Piety Instrument	0.71	

Note: *referent set to 1.

As Table 3 indicates, of the nine hypotheses that were proposed, six were supported by the data while three were not. The model relating the factor of role demands to caregiver levels of burden proved to be most helpful in interpreting our results. Of the three factors that influence caregiver burden, role demand is the most important ($\beta = .937$, $p < .05$). An increase in the caregiver's role demands increases the amount of caregiver burden. This finding suggests that the caregivers who are involved in several activities, such as working outside the home and taking care of chores within, experience a greater burden.

One significant source of role demand is the extent of caregiving tasks performed by the caregivers. South Asian caregivers who undertake extensive caregiving tasks experience high role demands ($\beta = .583$; $p < .05$), as expected. The level of caregiving tasks has a positive influence on their overall burden, as mediated and explained through the impact

on role demands (see Table 3). In turn, as the level of caregiving tasks increases, the complexity of role demand increases. Two significant sources of reduction in the pressures of role demands are size of the network and the extent of identification with Asian filial piety norms. These two background cultural factors are positively related. South Asian caregivers who have a large social support network of extended family and friends tend, also, to have fewer role demands placed on them. Thus, the size of the support network indirectly influences caregiver burden negatively through role demands. See Table 3. In addition, as expected, respondents who identified with traditional Asian filial piety norms of providing in-home care to their elderly parents tended to report fewer role demands ($\beta = -.241; p < .05$). Thus, we conclude that an increase in role demand significantly increases caregiving burden.

Table 3. Maximum Likelihood Estimates of the Determinants of Caregiver Burden in South Asian Families in the Dallas-Fort Worth Metropolitan area

Dependent Variable	Independent Variables	b	β	R-Square
Caregiver Burden	Role Demand	0.795	0.973*	0.71
	Caregiving Tasks	0.091	0.158	
	Support Provided by Elder	-.037	-.046	
Role Demand	Caregiving Tasks	0.396	0.583*	0.44
	Asian Cultural Identification	-.090	-.241	
	Size of Support Network	-.090	-.241	
Caregiving Tasks		0.074	0.126	0.02
Support Provided by Elder	Asian Cultural Identification	-.387	-.372*	
Size of Support Network	Role Demand			0.14
	Identification with Asian Filial Piety Norms	0.195	0.184	
*p<0.05				

The explanation of caregiving tasks proved less helpful. This hypothesis suggested that as the extent of caregiving tasks increases, the caregiver's level of burden would also

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increase. This expectation was not supported. This is possibly because the elders in this sample were relatively young and did not need a great deal of assistance. The average age of the care recipients was about 71 years, only seven of whom required ADL assistance. Caregiving tasks mostly included IADL such as financial care, buying groceries, and recreational and social activities. A large majority of the care recipients had no English language skills, could not drive, and therefore could not avail themselves of existing senior citizen services.

The elder support explanation indicated that as the amount of support provided by the elder increases, the amount of caregiver burden decreases. This hypothesis was also not supported. In multi generational South Asian households, elders provide several types of support. It was expected that as caregivers' role demands increased, the elderly were likely to increase their level of support in the household. There was a significant negative relationship in this regard ($\beta = -.372$; $p < .05$).

DISCUSSION

South Asian elderly parents who join their children in the United States are seldom familiar with local cultural and social settings. They often lack the ability to assess the external demands on the caregiver and may become unable to respond to increases in the number and complexity of the roles that caregivers play. The results of this study show that the social organization of caregiving among South Asians has two important cultural foundations: *Asian filial piety norms* and the *size of the social support network*. These cultural factors contribute to the role demands that caregivers experience. The impact of cultural variables is mediated by the degree of role demand. The contributions of local South Asian social support groups who share similar cultural values and norms remain effective in reducing caregiver burden (Giles et al., 2003).

We found that role demand is a crucial intervening factor in explaining caregiver burden. Of the three proposed explanations, the role demand explanation received the strongest empirical support. The extent of caregiving tasks performed by the caregiver is a significant factor in raising the level of caregiver burden insofar as it causes an increase in role demands. Furthermore, cultural factors play a significant role, even if indirect, in shaping the levels of caregiver burden. Both the size of the social support group and identification with filial piety norms indirectly reduce the level of perceived caregiver burden by reducing the amount of role demand.

Providing eldercare elevates the level of role demand and results in an increase in caregiver burden. In first generation South Asian families, cultural norms and networks associated with elder care giving, reduces the level of caregiver burden by first reducing the level of role demand. South Asians have established networks of extended family and friends in the Dallas-Fort Worth area. These help the caregivers by providing them with emotional, informational, and psychological support.

SOCIAL SERVICE POLICY IMPLICATIONS

The findings of this study reveal a pattern of stress and strain on South Asian families

attempting to live up to the demands of their culture while caring for elders in their own homes. The research also underscores the importance of enabling South Asians to preserve cultural values and networks to as they seek to reduce eldercare burden.

The immediate effects of the many role demands placed on the main caregivers shouldering these responsibilities raises serious policy concerns, with implications reaching beyond this particular immigrant community. These households provide many important services, such as preparation of food and the maintenance of clean clothing and a habitable environment. The production, distribution, and consumption of services in varying quantities by different members of the household require efficient allocation of both financial resources and time. One strategy for reducing the pressures on caregivers involves the coordination and support in the performance of these activities. Such a strategy would reduce inefficient use of time and financial resources.

Social service workers can assist households in reorganization, allocation, and planning of activities to reduce role demand. To lessen the role demands, several educational strategies may be effective in reducing role demands. These include learning to manage time and finances effectively and developing effective communication skills. Service providers may assist caregivers in dealing with role conflict and role overload (Gupta, 2000). The availability and access to services such as health and transportation for the elderly should also be considered as a potential resource (Rajan, Mishra, and Sarma, 2001; Valcour et al., 2002).

The reorganization of household activities necessarily involves household decision making, which is often based on familiar values, norms, and expectations about quality of life. The decision-making process is highly influenced by the current configuration of status, roles, and power distribution within the family. Social service workers should be knowledgeable about South Asian cultural values associated with elder caregiving.

We have found that cultural values influence the amount of role demand. This highlights the importance of a culturally sensitive approach to reducing role demand. Because of the hierarchical nature of power relations and decision making in South Asian households, it is inappropriate to involve children, caregivers, and elder parents in an open discussion. Instead, service providers should disseminate information about role management and role adjustments to caregivers and other members of the family through public lectures and small group discussions (Segal, 1991). This approach is preferable, as caregivers may not want to make personal admissions regarding the burden associated with caring for their elderly at home.

Provision of effective individual as well as group level counseling for dealing with role demands requires knowledge of South Asian cultural values and norms. The current counseling and therapy programs do not employ counselors or providers who are knowledgeable about South Asian cultural values and norms. As Ramisetty-Mikler (1993) points out, Western counseling modes that rely on introspection and clear verbalization of feeling and emotions often do not suit the needs of South Asian clients. She recommends the use of direct role-play techniques and assertive training programs in lieu of attempting to change belief systems and attitudes.

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Programs for South Asian families should be culturally sensitive, in accord with the social service orientation that emphasizes the need to "start where the client is." Service providers need to assist caregivers in maintaining cohesiveness within the family. Educational seminars that promote family caregiving should be provided to the community. The role that social networks play should be clearly understood and preserved. Defining the South Asian family as a client engaged in elder caregiving involves identifying the overlapping helping roles members play in their social networks as well as in their families.

The caregivers who participated in this study were affiliated with a number of religious and South Asian nonprofit organizations that promote cultural activities. Their affiliation enables many caregivers to share ideas and to cope with the stress of caregiving. Because of reductions in Federal Government expenditures, caregivers have relied on their friends and other professionals in the community to provide volunteer support for educational and informational services.

Government programs for South Asian elders should clearly build on the existing ties in community social support groups. Such programs need to reach out to volunteers, friends, and relatives who can help relieve the role demands on the primary caregivers. Access to these informal groups can be gained in consultation with community leaders. The employment of ethnic consultants is an effective way of lowering the costs of these programs.

NOTES

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1. Such households increased from 2 percent in 1970 to about 8 percent in 1998. The percentage figure is expected nearly to double within the next decade (Celeste, 1998).
2. There are approximately twenty thousand Indian and twenty thousand Pakistani persons of Asian descent residing in the Dallas-Fort Worth area (Bharati, 1996; Personal communication with Pakistani community leaders).
3. The final sample may not be representative of the population of South Asian caregivers in the Dallas/Fort Worth Area. South Asians consider familial matters such as caregiving as private, and they hesitate to discuss caregiver problems with outsiders. As a result, it is difficult to obtain cooperation from the community for undertaking scientific studies on caregiver burden. Incentives such as providing free educational programs were used to elicit cooperation from the community. These yielded only limited access to the community. Religious and non-profit organizations actively participated in the study. Thus, this sample probably over-represents populations belonging to religious organizations and might have under-represented those who do not belong to such institutions. For these reasons, our abilities to generalize the results of this study are limited.

4. SEM analysis can be implemented even when the sample size is not very large. According to Kerlinger and Pedhazur (1973: 446) "any multivariate analysis, and those with many independent variables, should have at least 100 subjects, although 200 or more would be preferable."

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